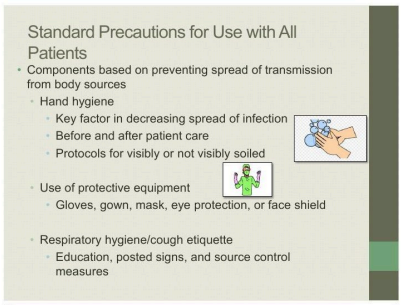
Head-to-Toe Check-off: Beginner Step-by-Step Guide (v3.4)

NRSG 233 Health Assessment, LLU, Fall 2026. This is a personal practice guide, not an assignment. It follows her **Head to Toe Skills Performance Rubric (63 pts)** exactly, in rubric order. Everything here is graded, and nothing else is included.

The rules (syllabus + rubric): Check-off in Week 9 (Nov 16–27), on a partner, recorded. **15-minute limit. No prompting:** −1 pt each time. You must score at least **76%** on this check-off to pass the course (a retake after mandatory practice caps at 80%).
How each assessment is scored (1.5 pts): 0.25 for saying the NAME of the assessment out loud, **0.75** for the correct TECHNIQUE, **0.5** for stating FINDINGS in medical terms. So for every item: **1) Say what you're doing, 2) do it right, 3) say the normal finding out loud.**
Points: Introduction/Safety 6 × 0.25 = 1.5. Assessments 41 × 1.5 = 61.5. **Total 63.** +1 bonus for the pulse grading scale.
Technique words: INSPECT = look (and listen) carefully. PALPATE = feel with your fingers or hand. PERCUSS = tap to hear the tone. AUSCULTATE = listen with the stethoscope on bare skin.
Sources: her slides first (Module 1, Mental Status, Module 2 HEENT, Module 3 Skin, Module 4 Abdomen & MS). Jarvis 9e page numbers come from her doc sheets and the rubric's note to "refer to Jarvis." Photos labeled Jarvis are textbook photos. The two diagrams were drawn for this guide.

Introduction / Safety (6 × 0.25 = 1.5 pts)

Do these the moment you start, in this order.



Her Module 1 slides: Standard Precautions

Washed Hands — 0.25

How: Walk in, go straight to the sink or foam dispenser. Rub all surfaces for at least 20 sec, then let it dry. Do it where the instructor can see.

Say: (Say it out loud): "I'm performing hand hygiene."

Common mistake: Starting to talk or touch the partner first. Doing it off-camera.


Jarvis p. 770

Introduced Self — 0.25

How: Face the partner at eye level and make eye contact.

Say: "Hi, my name is Luke. I'm a nursing student, and I'll be doing your physical assessment today."

Common mistake: Forgetting your role ("nursing student").


Jarvis p. 770

Identified Patient with 2 Identifiers — 0.25

How: Ask open questions. Don't say their name for them. Check against the armband or chart.

Say: "Can you please tell me your full name and date of birth?"

Common mistake: Asking "Are you John?" (that's a yes/no question). Using only one identifier.



Jarvis p. 770

Verified Allergy Status — 0.25

How: Ask, then confirm with the armband or chart.

Say: "Do you have any allergies to medications, foods, or latex?"

Common mistake: Skipping it. It's an easy 0.25.



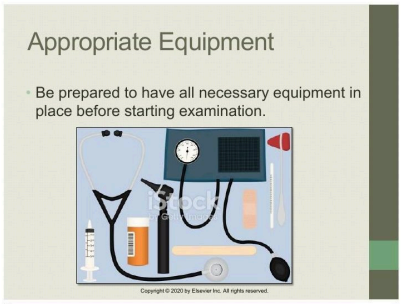
Jarvis p. 770

Explained Assessment to Patient — 0.25

How: Short and simple. Say they can stop you at any time.

Say: "I'm going to do a head-to-toe assessment. I'll look, listen, and feel from your head to your toes. It takes about 15 minutes. Let me know if anything is uncomfortable. Is that okay?"

Common mistake: Not asking for permission.



Her Module 1 slides: Appropriate Equipment

Cleaned Stethoscope — 0.25

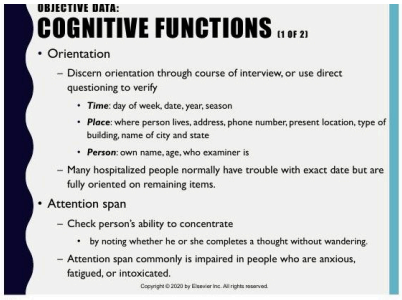
How: Wipe the diaphragm, bell, and earpieces with an alcohol pad before you use it. Let it dry.

Say: "I'm cleaning my stethoscope with an alcohol wipe."

Common mistake: Cleaning it silently. Say it so it gets graded.

Mental Status (3 × 1.5 = 4.5 pts)

Technique: INSPECT (watch and listen, no touching)
Position / where you stand: Partner sits on the bed. You sit or stand facing them at eye level.



Her Mental Status slides: Cognitive functions

1. Orientation (3 assessments) — 1.5

① **Name it (0.25):** "I'm assessing orientation to person, place, and time."
② **Do it (0.75):** Ask 3 questions, one each for person, place, and time. Listen to whether the answers are correct and quick.

Say to partner: "What is your name? Where are we right now? What is today's date?"

③ **State findings (0.5):** "Patient is alert and oriented ×3 to person, place, and time." (A&O×3)

Common mistake: Asking only 2 questions, or saying "A&O" without the ×3.

[Her slides; doc sheet: Jarvis p. 63]



Her Mental Status slides: Cultural assessment

2. Culture (1 assessment) — 1.5

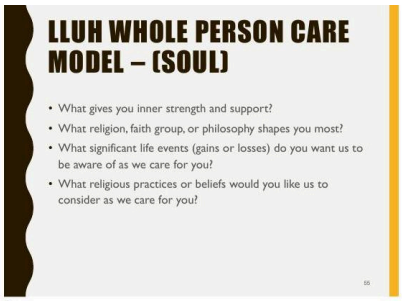
① **Name it (0.25):** "I'm doing a cultural assessment."
② **Do it (0.75):** Ask one open-ended question about cultural background or health practices.

Say to partner: "Is there anything about your culture or background I should know to care for you?"

③ **State findings (0.5):** "Patient identifies as ___. No cultural practices affecting care." (State what they say.)

Common mistake: Assuming their culture. Always ask.

[Her slides; doc sheet: Jarvis pp. 16–17]



Her slides: LLUH Whole Person Care (SOUL)

3. Spiritual (1 assessment) — 1.5

① **Name it (0.25):** "I'm doing a spiritual assessment."
② **Do it (0.75):** Ask one SOUL question (Sources of strength, Organized religion, Unique life events, Life practices).

Say to partner: "What gives you inner strength and support?"

③ **State findings (0.5):** "Patient states ___ is a source of strength. No spiritual needs at this time."

Common mistake: Pushing your own beliefs or asking a yes/no question.

[Her slides; doc sheet: Jarvis p. 17]

Head (1 × 1.5 = 1.5 pts)

Technique: INSPECT & PALPATE
Position / where you stand: Partner sits. Stand in front of them.


Jarvis p. 771 (photo)

4. Skull (size & symmetry) — 1.5

① **Name it (0.25):** "I'm inspecting and palpating the skull for size and symmetry."
② **Do it (0.75):** Look at the head from the front and sides. Then use the pads of your fingers to move the scalp in small circles, front to back, feeling for lumps or tenderness.

Say to partner: "I'm going to feel your head. Tell me if anything is tender."

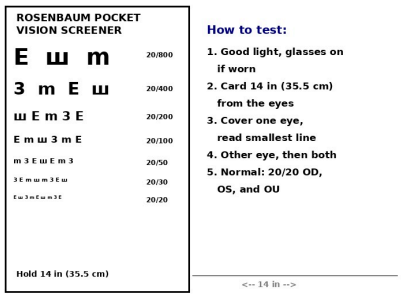
③ **State findings (0.5):** "Skull is normocephalic and symmetric. No lumps, lesions, or tenderness."

Common mistake: Only looking (you must also palpate), or forgetting the word "normocephalic."

[Jarvis p. 259; Module 2 slides]

Eyes (5 × 1.5 = 7.5 pts)

Technique: INSPECT
Position / where you stand: Partner sits. You stand in front. For acuity, they hold the Rosenbaum pocket card 14 inches (35.5 cm) from their eyes.



Rosenbaum near-vision pocket card (drawn diagram)

5. Visual Acuity — 1.5

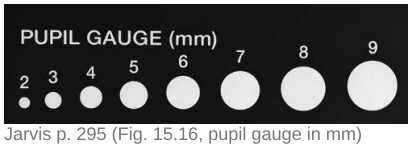
① **Name it (0.25):** "I'm testing visual acuity with the Rosenbaum chart."
② **Do it (0.75):** First ask: "Do you wear glasses or contacts? If so, please keep them on for this test." (Note whether the result is corrected.) In good light, partner holds the Rosenbaum near-vision card 14 inches (35.5 cm) from the eyes. Have them cover one eye and read the smallest line they can (one eye only for check-off). Full exam: repeat with the other eye, then with both eyes.

Say to partner: "Do you wear glasses or contacts? If so, please keep them on. Hold this card about 14 inches from your eyes. Cover your left eye and read the smallest line you can."

③ **State findings (0.5):** "Visual acuity 20/20 OD (one eye) by Rosenbaum chart, with/without correction, no squinting." (Full exam: "20/20 OS, OD, and OU by Rosenbaum chart, no squinting.") Note: Jarvis normal is 14/14 each eye; say the value your lab card uses (20/20 equivalent).

Common mistake: Forgetting to ask about glasses/contacts. Partner pressing on the covered eye. Holding the card too close or too far.

[Jarvis pp. 290–291; p. 288 (glasses/contacts question)]



Jarvis p. 295 (Fig. 15.16, pupil gauge in mm)

6. Pupil Size & Shape — 1.5

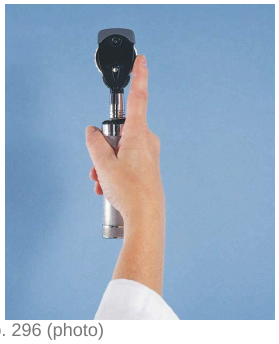
① **Name it (0.25):** "I'm inspecting pupil size and shape."
② **Do it (0.75):** Look at both pupils in normal room light and compare them.

Say to partner: "Look straight ahead at the wall."

③ **State findings (0.5):** "Pupils are equal, round, about 3–5 mm."

Common mistake: Not comparing the two sides.

[Her Module 2 slides; Jarvis p. 295]


Jarvis p. 296 (photo)

7. Direct Light Reflex

1.5

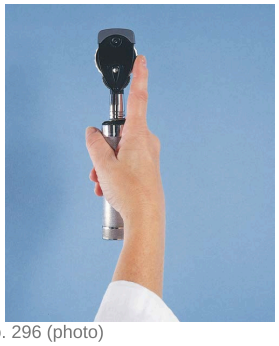
① **Name it (0.25):** "I'm testing the direct light reflex."
② **Do it (0.75):** Dim the light if you can. Bring the penlight in from the side (not straight on) into the RIGHT eye: the right pupil constricts (direct, right). Then repeat shining into the LEFT eye: the left pupil constricts (direct, left).

Say to partner: "Keep looking at the wall. I'm going to shine a light in your eye."

③ **State findings (0.5):** "Right pupil: brisk direct response to light. Left pupil: brisk direct response to light."

Common mistake: Shining it straight on, which makes them focus on the light.

[Her Module 2 slides; Jarvis p. 295]


Jarvis p. 296 (photo)

8. Consensual Light Reflex

1.5

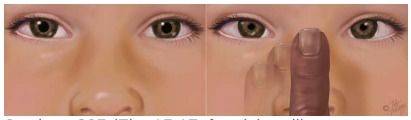
① **Name it (0.25):** "I'm testing the consensual light reflex."
② **Do it (0.75):** Shine the light into the RIGHT eye and watch the LEFT pupil constrict (consensual, left). Repeat shining into the LEFT eye and watch the RIGHT pupil constrict (consensual, right).

Say to partner: "Keep looking ahead."

③ **State findings (0.5):** "Right stimulus: left pupil brisk consensual response. Left stimulus: right pupil brisk consensual response."

Common mistake: Watching the wrong eye.

[Her Module 2 slides; Jarvis p. 295]


Jarvis p. 295 (Fig. 15.17: far vision dilates, near vision constricts)

9. Accommodation

1.5

① **Name it (0.25):** "I'm testing accommodation."
② **Do it (0.75):** Have them look at the far wall, then quickly at your finger held about 7–8 cm from their nose. The pupils should constrict and the eyes converge.

Say to partner: "Look at the wall... now look at my finger."

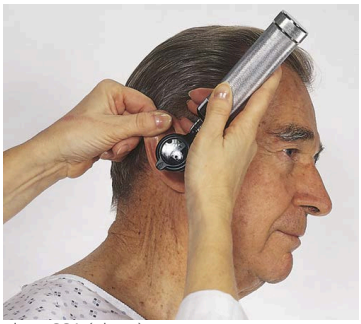
③ **State findings (0.5):** "Accommodation present: pupils constrict and eyes converge. PERRLA."

Common mistake: Holding your finger too far away.

[Her Module 2 slides; Jarvis p. 295]

Ears (2 × 1.5 = 3 pts)

Technique: INSPECT; INSPECT & PALPATE
Position / where you stand: Partner sits. Stand at their side.


Jarvis p. 331 (photo)

10. External Auditory Meatus

1.5

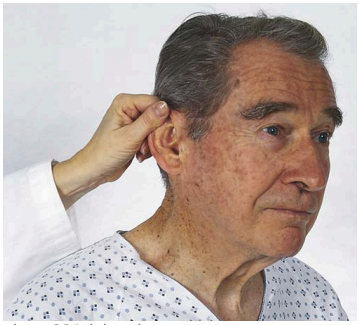
① **Name it (0.25):** "I'm inspecting the external auditory meatus."
② **Do it (0.75):** Look at the opening of the ear canal (penlight or otoscope). Note its size. If using the otoscope on an adult, gently pull the auricle (pinna) up and back.

Say to partner: "I'm going to look inside your ear."

③ **State findings (0.5):** "External auditory meatus without swelling, redness, or discharge."

Common mistake: Pulling the ear down (that's for children under 3).

[Jarvis pp. 330–331]


Jarvis p. 331 (photo)

11. External Ear

1.5

① **Name it (0.25):** "I'm inspecting and palpating the external ear."
② **Do it (0.75):** Look at the size, shape, and skin of both ears. Then palpate: move the auricle (pinna), push on the tragus, and press the mastoid process behind the ear.

Say to partner: "Tell me if this hurts."

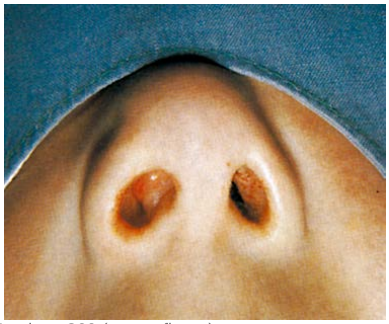
③ **State findings (0.5):** "Ears equal in size bilaterally, skin color consistent with facial skin, intact, no lumps or lesions. Pinna and tragus firm, nontender; mastoid nontender."

Common mistake: Only looking (you must also palpate).

[Jarvis p. 330]

Nose (2 × 1.5 = 3 pts)

Technique: INSPECT
Position / where you stand: Partner sits. Stand in front.


Jarvis p. 360 (nares, figure)

12. External Nose

1.5

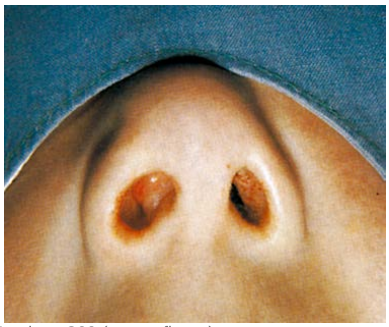
① **Name it (0.25):** "I'm inspecting the external nose."
② **Do it (0.75):** Look only (no touching). Check symmetry and midline position, shape and proportion to the face, skin color and lesions, any deformity or swelling, the nares (equal, open), any discharge, and nasal flaring.

Say to partner: "I'm going to look at your nose. Look straight ahead."

③ **State findings (0.5):** "External nose is symmetric and midline, proportional to facial features. Skin intact, no lesions or deformity. Nares symmetric, no discharge, no flaring."

Common mistake: Touching/palpating when only inspection is required. Forgetting to mention discharge and flaring.

[Her rubric (Nose: Inspect); Jarvis p. 360]


Jarvis p. 360 (nares, figure)

13. Patency of Nostrils

1.5

① **Name it (0.25):** "I'm testing patency of the nostrils."
② **Do it (0.75):** Press one nostril closed with your finger. Have them close their mouth and sniff. Then do the other side.

Say to partner: "Close your mouth and sniff in through your nose."

③ **State findings (0.5):** "Both nares are patent."

Common mistake: Closing both sides at once.

[Jarvis p. 360]

Mouth (4 × 1.5 = 6 pts)

Technique: INSPECT
Position / where you stand: Partner sits. Use a penlight and tongue blade. Put on gloves.


Jarvis p. 362 (photo)

14. Lips

1.5

① **Name it (0.25):** "I'm inspecting the lips."
② **Do it (0.75):** Look at the color, moisture, and any cracking or lesions. Gently retract the lips and look at the inner surface too.

Say to partner: "Relax your lips for me."

③ **State findings (0.5):** "Lips pink, moist, smooth, and symmetric. No cracking or lesions."

Common mistake: Forgetting to say the color.

[Jarvis p. 362]


Jarvis p. 362 (photo)

15. Gums

1.5

① **Name it (0.25):** "I'm inspecting the gums."
② **Do it (0.75):** Gloves on. Use the tongue blade to move the lips and cheeks aside and shine the light along the gum line.

Say to partner: "Open your mouth for me."

③ **State findings (0.5):** "Gums pink (coral) with stippled surface, margins tight and well defined. No swelling, bleeding, or retraction."

Common mistake: Doing it without gloves or light.

[Jarvis p. 362]



Jarvis p. 364 (photo)

16. Teeth

1.5

① **Name it (0.25):** "I'm inspecting the teeth."
② **Do it (0.75):** Look for diseased, absent, loose, or out-of-place teeth. Have them bite down to check alignment.

Say to partner: "Show me your teeth, then bite down like you're chewing."

③ **State findings (0.5):** "Teeth white, straight, evenly spaced, clean, free of debris or decay."

Common mistake: Forgetting to check the bite.

[Jarvis p. 362]



Jarvis p. 363 (photo)

17. Tongue

1.5

① **Name it (0.25):** "I'm inspecting the tongue."
② **Do it (0.75):** Have them stick out their tongue: check color, surface, and moisture. Then have them touch the roof of the mouth to see the underside.

Say to partner: "Stick out your tongue... now touch the roof of your mouth."

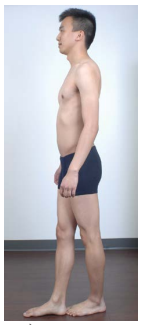
③ **State findings (0.5):** "Tongue pink and even, moist, midline; dorsal surface rough with papillae; ventral surface smooth and glistening. No lesions."

Common mistake: Forgetting the underside.

[Jarvis p. 363]

Motor System (2 × 1.5 = 3 pts)

Technique: INSPECT (gait); PALPATE (grip)
Position / where you stand: Gait: partner stands and walks. Grip: partner sits, and you stand in front.



Jarvis p. 647 (photo)

18. Gait

1.5

① **Name it (0.25):** "I'm inspecting gait."
② **Do it (0.75):** Have them walk across the room, turn, and walk back. Stay close for safety. Watch their posture, balance, arm swing, and stride.

Say to partner: "Please walk to the door, turn around, and come back. I'll stay right beside you."

③ **State findings (0.5):** "Gait is steady and coordinated, with even stride and arm swing."

Common mistake: Not staying close for safety.

[Jarvis pp. 646–647 (p. 646 is blank in the box PDF)]



Jarvis p. 658 (Fig. 24.40)

19. Grip

1.5

① **Name it (0.25):** "I'm palpating grip strength."
② **Do it (0.75):** Offer two fingers (one on top of the other) to each of their hands. Have them squeeze both hands at the same time and compare.

Say to partner: "Squeeze my fingers as hard as you can."

③ **State findings (0.5):** "Grip strength strong and equal bilaterally, 5/5."

Common mistake: Offering your whole hand (it can hurt). Not comparing sides.

[Her Module 4 slides (strength equal bilaterally, 0–5); Jarvis p. 658]

Sensory System (2 × 1.5 = 3 pts)

Technique: INSPECT
Position / where you stand: Partner sits with eyes CLOSED. Test both sides in matching spots (hands, feet).



Jarvis p. 649 (photo)

20. Light Touch

1.5

① **Name it (0.25):** "I'm testing light touch sensation."
② **Do it (0.75):** Lightly touch the skin with a cotton wisp in random spots on both sides.
Say to partner: "Close your eyes. Say 'now' every time you feel me touch you."
③ **State findings (0.5):** "Light touch sensation intact bilaterally."
Common mistake: Letting them watch. Using a predictable rhythm.
[Jarvis p. 649]



Jarvis p. 649 (photo)

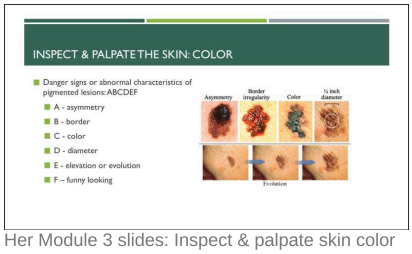
21. Pain

1.5

① **Name it (0.25):** "I'm testing superficial pain (sharp/dull)."
② **Do it (0.75):** Break a tongue blade lengthwise. Lightly touch with the sharp end and the dull end in a random, unpredictable order on both sides. Throw the blade away after.
Say to partner: "Eyes closed. Tell me 'sharp' or 'dull.'"
③ **State findings (0.5):** "Able to distinguish sharp and dull bilaterally."
Common mistake: Pressing too hard. Not mixing the order.
[Jarvis p. 649]

Skin (4 × 1.5 = 6 pts)

Technique: INSPECT (color); PALPATE (temp, edema, moisture)
Position / where you stand: Partner sits. Use their hands, arms, and shins.

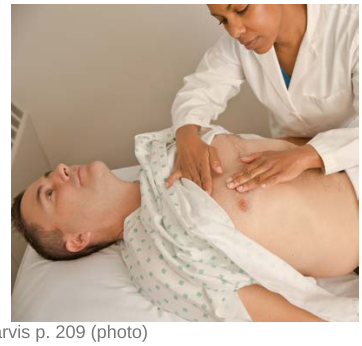


Her Module 3 slides: Inspect & palpate skin color

22. Color

1.5

① **Name it (0.25):** "I'm inspecting skin color."
② **Do it (0.75):** Look at the face, arms, hands, and nail beds in good light.
Say to partner: "Let me look at your arms and hands."
③ **State findings (0.5):** "Skin color even (homogeneous) and consistent with genetic background. No pallor, erythema, cyanosis, or jaundice."
Common mistake: N/A
[Her slides; doc sheet: Jarvis p. 207]

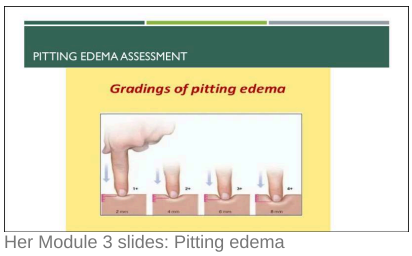


Jarvis p. 209 (photo)

23. Temperature

1.5

① **Name it (0.25):** "I'm palpating skin temperature."
② **Do it (0.75):** Use the BACK of your hand (it's most sensitive to temperature) on their arms and legs, comparing both sides.
Say to partner: "I'm going to feel your skin with the back of my hand."
③ **State findings (0.5):** "Skin warm and equal bilaterally."
Common mistake: Using your fingertips.
[Her slides; doc sheet: Jarvis pp. 209–210]

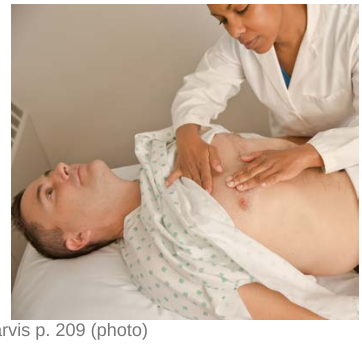


Her Module 3 slides: Pitting edema

24. Edema

1.5

① **Name it (0.25):** "I'm palpating for edema."
② **Do it (0.75):** Press firmly with your thumb over the shin (tibia) or behind the ankle bone (medial malleolus) for 5 seconds, release, and look for a dent. Do both legs.
Say to partner: "I'm going to press on your shin for a few seconds."
③ **State findings (0.5):** "No edema bilaterally." (If present: pitting or nonpitting; grade pitting 1+ mild to 4+ very deep.)
Common mistake: Pressing for only 1 second.
[Her slides; doc sheet: Jarvis pp. 210, 519]


Jarvis p. 209 (photo)

25. Moisture

1.5

① **Name it (0.25):** "I'm palpating skin moisture."
② **Do it (0.75):** Feel the skin surface with your fingertips; note moisture on the face, hands, axillae, and skinfolds.

Say to partner: N/A

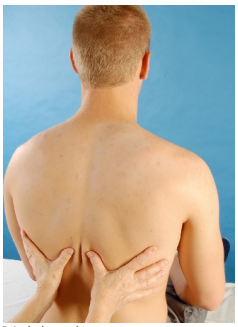
③ **State findings (0.5):** "Skin dry with slight perspiration only where expected. No diaphoresis or excessive dryness."

Common mistake: N/A

[Her slides; doc sheet: Jarvis p. 210]

Thorax & Lungs (Posterior Chest) (2 × 1.5 = 3 pts)

Technique: INSPECT; AUSCULTATE
Position / where you stand: Partner sits and leans slightly forward with arms crossed. Open the back of the gown. Stand behind them.


Jarvis p. 421 (photo)

26. Chest (shape & configuration)

1.5

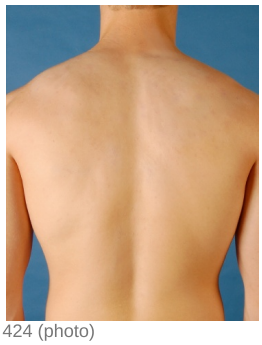
① **Name it (0.25):** "I'm inspecting the posterior chest for shape and configuration."
② **Do it (0.75):** Look at the spine, scapulae, ribs, and symmetry. Compare front-to-back (AP) depth to side-to-side (transverse) width.

Say to partner: "Please cross your arms and lean forward slightly."

③ **State findings (0.5):** "Thorax symmetric and elliptical, spinous processes in a straight line, scapulae symmetric, ribs slope downward about 45°. AP diameter less than transverse (ratio about 0.7–0.75)."

Common mistake: Forgetting to state the AP:transverse ratio.

[Jarvis p. 421]


Jarvis p. 424 (photo)

27. Chest (breath sounds)

1.5

① **Name it (0.25):** "I'm auscultating posterior breath sounds."
② **Do it (0.75):** Put the stethoscope diaphragm firmly on bare skin, between the ribs and away from the scapulae. Go side to side like a ladder, top to bottom (about 6 pairs), one full breath at each spot.

Say to partner: "Breathe in and out through your mouth, a little deeper than normal."

③ **State findings (0.5):** "Breath sounds clear bilaterally: vesicular over the peripheral lung fields, bronchovesicular between the scapulae. No adventitious sounds (no crackles or wheezes)."

Common mistake: Listening over the gown or over bone. Doing one whole side, then the other.

[Jarvis pp. 424–425]

Peripheral Vascular System (5 × 1.5 = 7.5 pts)

Technique: PALPATE; INSPECT & PALPATE
Position / where you stand: Partner sits for the carotid and radial pulses, then lies supine for the foot pulses.


Jarvis p. 471 (photo)

28. Carotid Pulses

1.5

① **Name it (0.25):** "I'm palpating the carotid pulses."
② **Do it (0.75):** Palpate each carotid artery medial to the sternomastoid muscle with 2 fingers. Avoid pressure higher in the neck (carotid sinus). Palpate gently, **ONE SIDE AT A TIME**.

Say to partner: "I'm going to feel the pulse in your neck."

③ **State findings (0.5):** "Carotid pulses 2+, smooth contour with brisk upstroke and slower downstroke, equal bilaterally."

Common mistake: Pressing both sides at once (it can cause fainting). Using your thumb.

[Jarvis p. 471]


Jarvis p. 513 (photo)

29. Radial Pulses

1.5

① **Name it (0.25):** "I'm palpating the radial pulses."
② **Do it (0.75):** Put 2–3 finger pads on the thumb side of the inner wrist. Compare both sides.

Say to partner: "Relax your arms."

③ **State findings (0.5):** "Radial pulses 2+, regular, elastic vessel wall, equal bilaterally."

Common mistake: Using your thumb.

[Jarvis p. 513]


Jarvis p. 519 (photo)

30. Dorsalis Pedis Pulses

1.5

① **Name it (0.25):** "I'm palpating the dorsalis pedis pulses."
② **Do it (0.75):** Feel on the top of the foot, just lateral to the big-toe tendon. Press lightly.

Say to partner: "I'm going to feel the pulse on top of your foot."

③ **State findings (0.5):** "Dorsalis pedis pulses 2+, equal bilaterally."

Common mistake: Pressing too hard, which blocks the pulse.

[Jarvis p. 519]


Jarvis p. 519 (photo)

31. Posterior Tibial Pulses

1.5

① **Name it (0.25):** "I'm palpating the posterior tibial pulses."
② **Do it (0.75):** Curl your fingers behind and just below the inner ankle bone (medial malleolus).

Say to partner: N/A

③ **State findings (0.5):** "Posterior tibial pulses 2+, equal bilaterally."

Common mistake: Feeling on the outer ankle.

[Jarvis p. 519]


Jarvis p. 215 (photo)

32. Capillary Refill

1.5

① **Name it (0.25):** "I'm inspecting and palpating capillary refill."
② **Do it (0.75):** With their hand near heart level, depress and blanch the nail bed, release, and note the time for color return. Do both hands. (Normal: within 1–2 seconds.)

Say to partner: "I'm going to press on your fingernail."

③ **State findings (0.5):** "Capillary refill less than 2 seconds bilaterally."

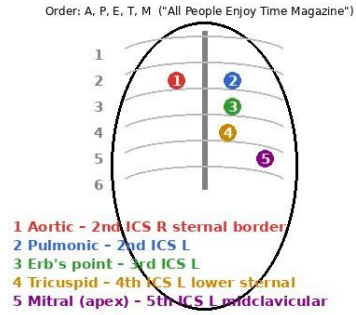
Common mistake: Testing with the hand hanging far below or raised above heart level.

[Jarvis p. 513; doc sheet: Jarvis p. 215]

★ **BONUS +1 pt: Pulse Grading Scale** [Jarvis p. 513]. Define each grade out loud: **0 = absent**; **1+ = weak / thready**; **2+ = normal**; **3+ = increased / full, bounding**. Say your pulses as "2+" so the grading is obvious.

Heart (5 × 1.5 = 7.5 pts)

Technique: AUSCULTATE
Position / where you stand: Partner supine with the head raised about 30° (or sitting). Stand on their RIGHT. Diaphragm on bare skin. Use the diagram: **APETM**.

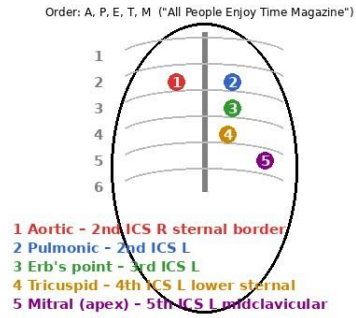


Drawn diagram (sites per Jarvis p. 475)

33. Aortic Area

1.5

① **Name it (0.25):** "I'm auscultating the aortic area."
② **Do it (0.75):** Point 1: 2nd intercostal space, RIGHT sternal border. Find the angle of Louis (the bump on the sternum), slide right into the 2nd ICS.
Say to partner: "Breathe normally. I'm going to listen to your heart."
③ **State findings (0.5):** "S1 and S2 heard, regular rate and rhythm, S2 louder at the base. No murmurs."
Common mistake: Starting on the left side.
[Jarvis pp. 475–477]

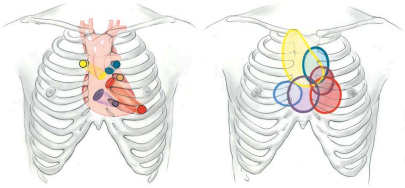


Drawn diagram

34. Pulmonic Area

1.5

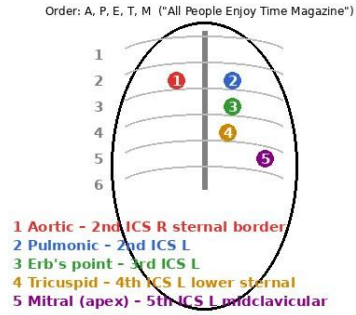
① **Name it (0.25):** "I'm auscultating the pulmonic area."
② **Do it (0.75):** Point 2: 2nd ICS, LEFT sternal border.
Say to partner: N/A
③ **State findings (0.5):** "S1 and S2 heard, regular. No murmurs."
Common mistake: N/A
[Jarvis pp. 475–477]


Jarvis p. 475 (figure)

35. Erb's Point

1.5

① **Name it (0.25):** "I'm auscultating Erb's point."
② **Do it (0.75):** Point 3: 3rd ICS, LEFT sternal border.
Say to partner: N/A
③ **State findings (0.5):** "S1 and S2 heard, regular. No murmurs."
Common mistake: N/A
[Jarvis pp. 475–477]



Drawn diagram

36. Tricuspid Area

1.5

① **Name it (0.25):** "I'm auscultating the tricuspid area."
② **Do it (0.75):** Point 4: LEFT lower sternal border (4th ICS).
Say to partner: N/A
③ **State findings (0.5):** "S1 and S2 heard, regular. No murmurs."
Common mistake: N/A
[Jarvis pp. 475–477]


Jarvis p. 476 (photo)

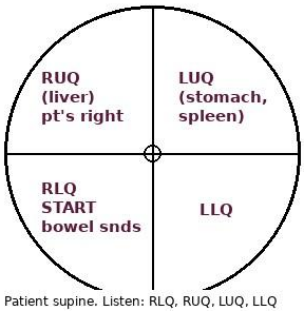
37. Mitral Area

1.5

① **Name it (0.25):** "I'm auscultating the mitral area (apex)."
② **Do it (0.75):** Point 5: 5th ICS at about the LEFT midclavicular line (the apex). For women, ask them to lift the breast slightly.
Say to partner: N/A
③ **State findings (0.5):** "S1 and S2 heard, regular rate and rhythm, S1 louder at the apex. No murmurs, gallops, or rubs."
Common mistake: Listening too close to the sternum.
[Jarvis pp. 475–477]

Abdomen (4 × 1.5 = 6 pts)

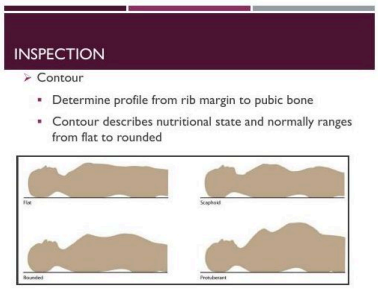
Technique: Order: INSPECT, AUSCULTATE, PERCUSS, PALPATE (listen before you touch!)
Position / where you stand: Partner supine and flat, arms at sides, knees slightly bent, bladder emptied. Drape. Stand on their RIGHT. Warm your hands and stethoscope.



Drawn diagram: the 4 quadrants (the patient's right is on YOUR left as you face them)

Quadrant map

Draw imaginary lines through the umbilicus, one vertical and one horizontal. RUQ = liver. LUQ = stomach and spleen. Bowel sounds start in the RLQ.



Her Module 4 slides: Inspection

38. Abdominal contour

1.5

① **Name it (0.25):** "I'm inspecting abdominal contour."

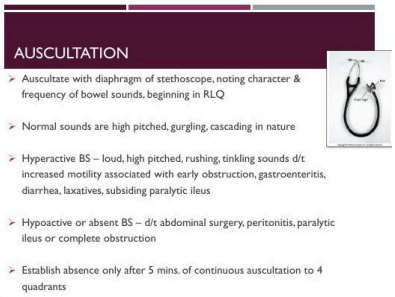
② **Do it (0.75):** Stand on their RIGHT and look down at the abdomen, then stoop so your eyes are just above it and look across from the rib margin to the pubic bone.

Say to partner: "Lie flat with your arms at your sides."

③ **State findings (0.5):** "Abdomen flat (or rounded) and symmetric. Umbilicus midline and inverted, no discoloration or inflammation. Skin smooth and even with homogeneous color. No visible pulsations."

Common mistake: Looking only from above.

[Her slides; doc sheet: Jarvis p. 542]



Her Module 4 slides: Auscultation

39. Bowel Sounds

1.5

① **Name it (0.25):** "I'm auscultating bowel sounds."

② **Do it (0.75):** Put the diaphragm lightly on the skin. **Start in the RLQ**, then RUQ, LUQ, LLQ (see the diagram on the abdomen page).

Say to partner: "I'm going to listen to your belly. Breathe normally."

③ **State findings (0.5):** "Bowel sounds present in all 4 quadrants: high-pitched, gurgling, cascading, irregular, 5–30/min." (Call them absent only after 5 min of continuous listening.)

Common mistake: Palpating before you listen. Starting in the wrong quadrant.

[Her slides; doc sheet: Jarvis p. 545]


Jarvis p. 547 (Fig. 22.13, percussing 4 quadrants)

40. Abdomen (general tone)

1.5

① **Name it (0.25):** "I'm percussing the abdomen for general tone."

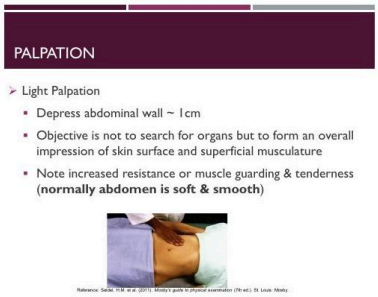
② **Do it (0.75):** Indirect percussion: put the middle finger of your non-dominant hand flat on the skin (only that finger touching). Tap its last joint twice with the tip of your other middle finger, snapping from the wrist. Percuss lightly in all 4 quadrants, moving clockwise.

Say to partner: "You'll feel some tapping."

③ **State findings (0.5):** "Tympany predominates in all 4 quadrants."

Common mistake: Resting your whole hand on the abdomen, which muffles the sound. (Her slides: don't do liver span.)

[Jarvis pp. 546–547; her Module 4 slides]



Her Module 4 slides: Light palpation

41. Abdomen (all 4 quadrants)

1.5

① **Name it (0.25):** "I'm performing light palpation of all 4 quadrants."

② **Do it (0.75):** Use the pads of 4 fingers together and press only about 1 cm in, with gentle circles, through all 4 quadrants, lifting (not dragging) your fingers and moving clockwise. Watch their face. Do any tender spot last.

Say to partner: "Tell me if anything is tender. I'll be gentle."

③ **State findings (0.5):** "Abdomen soft and smooth in all 4 quadrants. No tenderness, guarding, or rigidity."

Common mistake: Pressing too deep (light is about 1 cm). Not watching their face.

[Her slides; doc sheet: Jarvis p. 548]

Closing (not graded, but do it)

Help the partner sit up or get comfortable, put the bed down, place the call light within reach, thank them, wash your hands, and say: "We're all done. Here's your call light." [Jarvis p. 779]

One-page checklist (rubric order). For every item: NAME it, DO it, STATE findings. 15 min, no prompting.

#	Topic	Item	Pts	✓
	Intro/Safety	Washed Hands	0.25	☐
	Intro/Safety	Introduced Self	0.25	☐
	Intro/Safety	Identified Patient with 2 Identifiers	0.25	☐
	Intro/Safety	Verified Allergy Status	0.25	☐
	Intro/Safety	Explained Assessment to Patient	0.25	☐
	Intro/Safety	Cleaned Stethoscope	0.25	☐
1	Mental Status	Orientation (3 assessments)	1.5	☐
2	Mental Status	Culture (1 assessment)	1.5	☐
3	Mental Status	Spiritual (1 assessment)	1.5	☐
4	Head	Skull (size & symmetry)	1.5	☐
5	Eyes	Visual Acuity (one eye; ask glasses/contacts, keep on, note corrected)	1.5	☐
6	Eyes	Pupil Size & Shape	1.5	☐
7	Eyes	Direct Light Reflex (R eye then L eye, same pupil)	1.5	☐
8	Eyes	Consensual Light Reflex (light R, watch L; light L, watch R)	1.5	☐
9	Eyes	Accommodation	1.5	☐
10	Ears	External Auditory Meatus	1.5	☐
11	Ears	External Ear	1.5	☐
12	Nose	External Nose (inspect only)	1.5	☐
13	Nose	Patency of Nostrils	1.5	☐
14	Mouth	Lips	1.5	☐
15	Mouth	Gums	1.5	☐
16	Mouth	Teeth	1.5	☐
17	Mouth	Tongue	1.5	☐
18	Motor System	Gait	1.5	☐
19	Motor System	Grip	1.5	☐
20	Sensory System	Light Touch	1.5	☐
21	Sensory System	Pain	1.5	☐
22	Skin	Color	1.5	☐
23	Skin	Temperature	1.5	☐
24	Skin	Edema	1.5	☐
25	Skin	Moisture	1.5	☐
26	Thorax & Lungs	Chest (shape & configuration)	1.5	☐
27	Thorax & Lungs	Chest (breath sounds)	1.5	☐
28	Peripheral Vascular System	Carotid Pulses	1.5	☐
29	Peripheral Vascular System	Radial Pulses	1.5	☐
30	Peripheral Vascular System	Dorsalis Pedis Pulses	1.5	☐
31	Peripheral Vascular System	Posterior Tibial Pulses	1.5	☐
32	Peripheral Vascular System	Capillary Refill	1.5	☐
33	Heart	Aortic Area	1.5	☐
34	Heart	Pulmonic Area	1.5	☐
35	Heart	Erb's Point	1.5	☐
36	Heart	Tricuspid Area	1.5	☐
37	Heart	Mitral Area	1.5	☐
38	Abdomen	Abdominal contour	1.5	☐
39	Abdomen	Bowel Sounds	1.5	☐
40	Abdomen	Abdomen (general tone)	1.5	☐
41	Abdomen	Abdomen (all 4 quadrants)	1.5	☐
	Bonus	Pulse grading scale: 0, 1+, 2+, 3+	+1	☐
Total			**63**	